

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

for the administration of Miconazole 2% cream for the treatment of Fungal Skin Infections, and Trimovate cream for inflamed intertrigo or infected eczema with a suspected secondary bacterial or candidal component

Patient Group Direction, version 002, issued 10 September 2026. Supersedes Version 001, 1 November 2025.

Summary of NICE / NICE CKS Guidelines for Fungal Infections

Overview

- Fungal infections are usually caused by **dermatophytes**, **yeasts** (e.g. *Candida*), or **non-dermatophyte moulds**.
- Most are **superficial**, affecting **skin**, **nails**, or **mucosa**, and are generally **non-serious but can be persistent**.

**1. Tinea (Dermatophyte) Infections

Tinea Corporis (body), Cruris (groin), Pedis (foot), Manuum (hand):

- **First-line: Topical antifungals** (e.g. **clotrimazole**, **terbinafine**, **ketoconazole**), apply **once or twice daily for 2–4 weeks**.
- Use **oral antifungals** (e.g. **terbinafine**, **itraconazole**) if:
 - o Extensive/severe infection
 - o Failed topical therapy
 - o Immunosuppression

Tinea Capitis (scalp):

- Requires **oral antifungal treatment**:
 - o **Terbinafine** (first-line) or **griseofulvin** (children)

- o Adjunct **medicated antifungal shampoo** (e.g. ketoconazole) to reduce spread
 - Consider **referral** for diagnosis confirmation and contact tracing in children (especially in schools)
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2. Candidiasis (Yeast Infections)

Oral Candidiasis:

- First-line: **Topical nystatin** or **miconazole gel**
- **Fluconazole** orally if:
 - o Severe
 - o Unresponsive to topical
 - o Immunocompromised

Vaginal Candidiasis:

- **Topical imidazole** (e.g. clotrimazole) or **oral fluconazole 150 mg single dose**
- Recurrent: Consider **fluconazole 150 mg every 72 hours for 3 doses**, then weekly for 6 months

Intertrigo or Skin Fold Candidiasis:

- **Topical antifungal with or without mild corticosteroid** (e.g. miconazole/hydrocortisone)
 - Address underlying moisture or friction
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When to Refer

- Severe, widespread, or resistant infections
 - Immunocompromised or diabetic patients
 - Diagnostic uncertainty or suspected non-fungal skin condition
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General Advice

- Continue treatment for **at least 1–2 weeks after resolution**.
- Keep skin clean and dry.
- Avoid sharing towels, socks, footwear.
- Consider screening or treating close contacts in tinea capitis or extensive infection.

Patient Group Direction

for the supply/administration of

Miconazole 2% cream

for the treatment of

Fungal Skin Infection

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	● Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must previously have used PGD's to supply medication ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Miconazole 2% cream is indicated for the treatment of superficial fungal skin infections such as athlete's foot, ringworm, and candidal intertrigo in individuals aged 16 years and over.
Inclusion criteria	- Individuals aged 16 years and over. - Diagnosis of superficial fungal skin infection (e.g. athlete's foot, ringworm). - Informed consent obtained. - No contraindications to miconazole.
Exclusion criteria	- Known hypersensitivity to miconazole or any of the excipients. - Infected, broken, or oozing skin where topical antifungal is inappropriate. - Nail or scalp infections (requires systemic or alternative treatment). - Pregnancy or breastfeeding unless approved by prescriber.
Cautions	- Avoid contact with eyes and mucous membranes. - Continue treatment for several days after resolution of symptoms. - Wash hands before and after application. - Counsel on hygiene to prevent reinfection or spread. - Caution should be exercised and anticoagulant effect should be monitored in patients on warfarin or other vitamin K antagonists.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Miconazole 2% cream
Legal category	POM
Storage	Store below 25°C. Do not freeze.
Route/method of administration	Topical application to clean, dry affected area twice daily. Rub the cream into the skin with your finger until it has fully penetrated.
Dose and frequency	Apply thinly to affected area twice daily. The duration of therapy can vary depending on the localisation and the severity of the lesion, but treatment should be continued for at least one week after disappearance of all signs and symptoms.
Quantity to be administered and/or supplied	One 30 g tube per treatment course.
Maximum or minimum treatment period	Minimum 2 weeks; continue for several days after symptoms resolve. Maximum 4 weeks.
Adverse effects	Burning, stinging, itching, redness. Rare: allergic reaction, contact

	dermatitis. Refer to SmPC.
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Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply dose, form and route quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	To seek medical advice if symptoms worsen rapidly or significantly, do not improve in 3–4 weeks, or they become systemically very unwell. Wear loose-fitting clothes made of cotton or material designed to keep moisture away from the skin. Maintain good hygiene and wash affected skin areas daily and dry thoroughly, especially in the skin folds. Avoid scratching affected skin, as this may spread the infection to other sites. Do not share towels, and wash towels, clothes and bed linen frequently to eradicate fungal spores and reduce the risk of transmission.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 <https://www.nice.org.uk/guidance/mpg2>
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 <https://www.nice.org.uk/guidance/mpg2/resources>
- NICE CKS Guidance

- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines [BNF \(British National Formulary\) | NICE](#)
- Summary of Product Characteristics for drug

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date		Expiry date	
10/9/26		31/7/27	

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		2/11/2025
Chris Pilkington	Pharmacist GPhC: 2046322		2/11/2025

Change history

Version number	Change details	Date
001	Development & issue of new PGD	1 st November 2025

Patient Group Direction

for the supply/administration of

Trimovate cream

for the treatment of

Inflammatory Fungal Skin Conditions

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	● Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must previously have used PGD's to supply medication ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Trimovate cream is indicated for short-term treatment of inflamed intertrigo, infected eczema or seborrhoeic dermatitis where a secondary bacterial or candidal component is suspected and combined topical therapy is appropriate. It is NOT a treatment for ringworm (tinea of any site), athlete's foot or any other primary fungal infection: those are treated with miconazole under the first arm of this document, or referred.
Inclusion criteria	- Adults aged 18 years and over. - Localised inflamed skin condition with suspected fungal or bacterial involvement. - No signs of systemic infection. - Informed consent obtained.
Exclusion criteria	- Known hypersensitivity to corticosteroids, nystatin, oxytetracycline, or any excipients. - Skin infections requiring systemic therapy. - Broken, ulcerated, or weeping skin. - Use on face, genitals, or long-term use without review. - Pregnancy or breastfeeding unless deemed appropriate by prescriber. - Primary fungal infection of the skin (tinea of any site, including ringworm and athlete's foot), primary bacterial skin infection (impetigo, cellulitis), or viral skin infection (herpes simplex, chickenpox, shingles, warts, molluscum). Trimovate is contraindicated in all of these (SmPC 4.3).
Cautions	- Do not use under occlusion unless advised by prescriber. - Avoid prolonged use; assess if symptoms persist or worsen. - Avoid contact with eyes and mucous membranes. - Educate patient on appropriate use and hygiene to prevent spread. - If a patient presents with symptoms such as blurred vision or other visual disturbances, the patient should be considered for referral to an ophthalmologist for evaluation of possible causes.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Trimovate cream containing clobetasone 17-butyrate 0.05% w/w, oxytetracycline 3.0% w/w as calcium oxytetracycline and nystatin 100,000 units per gram.
Legal category	POM
Storage	Store below 25°C.
Route/method of	Topical application to affected area once or twice daily.

administration	Apply thinly and gently rub in using only enough to cover the entire affected area. Wash hands after applying unless it is the hands that are being treated. Allow adequate time for absorption after each application before applying an emollient if needed.
Dose and frequency	Apply thinly to affected area once or twice daily as directed.
Quantity to be administered and/or supplied	One 30 g tube per treatment course.
Maximum or minimum treatment period	Maximum 7–10 days continuous use without review. If the infection worsens, treatment and diagnosis should be re-evaluated as soon as possible. If the condition does not improve within seven days, treatment and diagnosis should be re-evaluated.
Adverse effects	Skin thinning, irritation, burning, itching. Rare: allergic reaction, superinfection. Refer to SmPC.

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply dose, form and route quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or	Supply the patient information leaflet (PIL) provided with the medication.
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carer	
Follow-up advice to be given to patient or carer	To seek medical advice if symptoms worsen rapidly or significantly, do not improve in 3–4 weeks, or they become systemically very unwell. Wash the affected skin daily and dry thoroughly, especially in skin folds. This helps to reduce the risk of reinfection. Wash clothes and bed linen frequently to eradicate the fungus. Avoid sharing towels and wash them regularly. Wear loose-fitting clothes made of cotton or materials designed to keep moisture away from the skin. Avoid scratching the affected skin as this may spread the infection.

Key references

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Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date

Name of professional group signatory	Job title & name of organisation	Signature	Date

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		2/11/2025
Chris Pilkington	Pharmacist GPhC: 2046322		2/11/2025

Change history

Version number	Change details	Date
001	Development & issue of new PGD	1 st November 2025

Version and change record

Version: v002

Issued: 10 September 2026

Supersedes: Version 001, 1 November 2025

Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

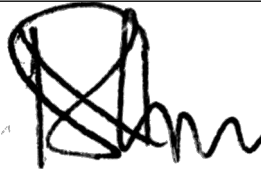
Changes in this version:

Validity: this version is valid from 10 September 2026 and expires on 31 July 2027. The signed master previously stated no period of validity, or an expiry of 31 October 2026; those are superseded.

Trimovate arm: the previous version framed Trimovate as a treatment for fungal skin infection and sat it under a guidance summary about tinea. The Trimovate SmPC contraindicates primary fungal, bacterial and viral skin infection. The indication now names inflamed intertrigo, infected eczema and seborrhoeic dermatitis with a suspected secondary component, states that Trimovate is not a treatment for ringworm or athlete's foot, and primary fungal, bacterial and viral infection are exclusions. The document title says which arm treats which.

Signed on behalf of Get Real Health

Version v002, 10 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		10 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		10 September 2026

Both authorising signatories reviewed this version together on 10 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.